Greater Western Hospital

Discharge Summary

55 Federation Street, Western Plains NSW 2830 | Tel: (02) 6883 1000 | Fax: (02) 6883 1001
Western NSW Local Health District | NSW Health

Patient Name:	**Hassan Nicholas Pham**	Medical Record No.:	**145635**
Date of Birth:	05/08/1963 (Age 62)	Medicare No.:	5586 71046 4
Address:	349 Castlereagh Pde, Chatswood NSW 2067	Next of Kin:	Robert Pham (Brother) 0452 497 341
GP:	Dr Olivia Chen, Tweed Coast Family Medicine	Allergies:	**Penicillin (rash), NSAIDs (asthma)**

ADMISSION DETAILS

Date Admitted:	07/05/2025 via Emergency Dept	Date Discharged:	09/05/2025
Ward:	8E (Geriatrics)	Length of Stay:	2 days
Admitting Clinician:	Dr P. Walker (PGY3 Registrar, General Medicine)	Consultant:	Dr R. Liu (Staff Specialist)
Discharge Destination:	Home (independent)	Discharge Mode:	Patient transport

PRESENTING COMPLAINT

Mr Pham was brought in with a 4-day history of progressive redness, warmth. Also, pain over the right lower leg. Reported a small skin break following gardening 1 week prior. Fevers to 38.5°C in the past 24 hours.

PRINCIPAL DIAGNOSIS

Cellulitis of right lower leg (ICD-10-AM: L03.115)

ADDITIONAL DIAGNOSES

1. Type 2 diabetes mellitus
2. Chronic venous insufficiency
3. Tinea pedis

HOSPITAL COURSE

Treated with IV flucloxacillin 2g 6-hourly with marked recovery of erythema and resolution of fevers within 48 hours. Switched to oral cefalexin to complete a 10-day total antibiotic course. Tinea pedis identified as likely portal of entry; treated with topical terbinafine. Diabetes optimised during admission with input from diabetes educator.

MEDICATIONS AT DISCHARGE

Medication	Dose / Route / Frequency	Notes
Cefalexin	500mg PO QID	NEW - 6 days remaining
Terbinafine	1% TOP daily	NEW - to feet for tinea pedis, 4 weeks
Metformin	1000mg PO BD	Unchanged
Atorvastatin	20mg PO nocte	Unchanged
Paracetamol	1g PO QID PRN	for pain

KEY INVESTIGATIONS

Bloods on admission (07/05/2025): WCC 13.6 x10^9/L (H), CRP 98 mg/L (H), Glucose 9.8 mmol/L (H).

Bloods at discharge (09/05/2025): WCC 8.2 x10^9/L, CRP 18 mg/L (H).

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Imaging: Doppler ultrasound right leg: no DVT. Soft tissue oedema consistent with cellulitis.

FOLLOW-UP PLAN

1. Complete remaining 6 days of oral cefalexin.
2. Re-present to ED if worsening erythema, fever, or systemic symptoms.
3. GP review at 1 week for clinical review and consideration of compression therapy.
4. Continue topical antifungal therapy for 4 weeks.

ALERTS AND PRECAUTIONS

ALLERGY: Penicillin (rash)
ALLERGY: NSAIDs (asthma)
Diabetes optimisation referral.

Foot care education provided to reduce risk of recurrence.

PREPARED BY

Dr Priya Walker, PGY3 Registrar, General Medicine

Reviewed and approved by: Dr Robert Liu, Staff Specialist, General Medicine

Date: 09/05/2025

Dr P. Walker (PGY3 Registrar) Dr R. Liu (Staff Specialist)

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